

NEW UNIT BRIEF

Charge Nurse Acuity and Assignment Platform

The platform gives a charge nurse one live workspace for staffing, acuity tags, patient movement, assignment balance, sitter coverage, discharge expectations, and shift handoff. It is built for the ordinary pressure of a hospital unit: rapid changes, incomplete information, interruptions, staffing variability, and the need to make fair decisions quickly.

One board

Current and oncoming assignments, patient details, room status, and acuity indicators in a shared view.

Faster balancing

Rebalance support for count fairness, acuity spread, room clustering, expected discharges, and continuity.

Safer handoff

Visible flags help reduce missed workload patterns before finalizing the next shift assignment.

What the platform is

This is an operational decision-support tool for charge nurses and unit leaders. It does not replace clinical judgment, staffing policy, or escalation pathways. Instead, it makes the assignment logic visible: which patients are high workload, where rooms are spread out, which nurses or PCAs may be stacked with discharges or acuity, and where a small adjustment may improve fairness.

1. Maintain beds

Room occupancy, acuity tags, expected discharges, sitter needs, and patient profile details.

2. Build assignments

Populate RN/PCA boards using active patients, staffing, rules, and continuity locks.

3. Rebalance safely

Find cleaner distributions while preserving pinned patients, sitter constraints, and role-specific rules.

4. Handoff clearly

Print or review a charge report with workload patterns, room spread, and acuity summaries.

Why it matters on a new unit

Consistency

New units need shared mental models.

Early in a rollout, staff are learning local workflows, room geography, handoff norms, and each other's strengths. A visible assignment system helps the unit standardize how workload is discussed.

Fairness

Fairness is more than patient count.

A group with four patients can be heavier than a group with five if it holds NIH checks, drips, isolation, sitter needs, strict I/O, feeders, or multiple expected discharges.

The goal is not to make the "perfect" assignment. The goal is to make the tradeoffs visible enough that the charge nurse can make a better, faster, more explainable decision.

CAPABILITIES

What it can do for daily unit flow

The platform supports high-frequency operational decisions: patient moves, profile edits, new admits, discharges, sitter assignments, staffing changes, and oncoming shift preparation.

Core functions

Acuity

Structured patient tags

Track RN and PCA-relevant workload drivers such as tele, NIH, drips, BG checks, isolation, sitter, total care, CHG, Foley, feeder, admit, and late discharge.

Assignments

Live and oncoming boards

Build current and next-shift RN/PCA assignments, drag patients between staff, and preserve continuity when clinically or operationally appropriate.

Rebalance

Workload review

Review count spread, load spread, room spread, report-source fragmentation, expected discharge stacking, and rule flags.

Sitter

Sitter constraints

Support sitter room-pair logic and prevent sitter-tagged patients from drifting into unsafe or confusing coverage patterns.

Handoff

Charge report

Create a concise report for the next shift showing patient distribution, relevant acuity clusters, discharge expectations, and assignment concerns.

Assistant

Virtual charge nurse lens

Ask about balance, room spread, acuity distribution, warning flags, tele/NIH rooms, feeder rooms, total care, and possible move ideas.

Operational value for staff

Less hidden work

Reduces the mental math burden.

Charge nurses often hold the entire assignment in working memory while also handling calls, admissions, staffing changes, and escalation. The platform externalizes that logic so the unit can see the same risk patterns.

Better conversations

Makes fairness explainable.

When staff ask why an assignment changed, the charge nurse can point to count spread, acuity load, room geography, discharge stacking, or continuity instead of relying on a subjective explanation.

Smoother starts

Improves shift-start clarity.

A cleaner oncoming board can reduce last-minute reshuffling, repeated report sources, and confusion about which staff member owns which patient.

Learning tool

Supports new charge nurses.

The system provides a repeatable framework for thinking through assignments, making it easier to teach charge decision-making without relying only on informal memory.

Implementation note: the platform should be introduced as a charge-nurse support tool, not as a punitive productivity tracker. Adoption is strongest when staff see it as a fairness, visibility, and safety aid.

SAFETY AND SATISFACTION RATIONALE

Why it can help reduce preventable risk

Preventable harm is rarely caused by one assignment decision alone. It often comes from accumulated operational friction: uneven workload, repeated interruptions, unclear ownership, long room runs, missed high-risk tags, and rushed handoff. The platform targets those upstream conditions.

How the platform supports staff satisfaction

Fair workload

Balances visible and invisible workload.

Staff satisfaction improves when assignments feel explainable and when high-burden tasks are not repeatedly stacked on the same person. The platform helps show that fairness is being actively reviewed.

Psychological safety

Creates a neutral reference point.

Instead of debating personalities or preferences, the unit can discuss the board: acuity tags, room spread, discharge load, patient count, and handoff complexity.

Reduced friction

Decreases repeated rework.

Faster reassignment and clearer oncoming boards can reduce the time staff spend renegotiating ownership after report has started.

Charge support

Protects the charge nurse's bandwidth.

By organizing assignment facts, the tool can reduce cognitive load and free the charge nurse to focus on escalation, coaching, patient flow, and real-time safety concerns.

How it can reduce preventable hospital-acquired injuries

- **Falls and sitter risk:** sitter tags, room-pair logic, and visible coverage reduce ambiguity around who is watching high-risk patients.
- **Pressure injury prevention:** PCA workload tags such as total care, turns, feeder, Foley, CHG, and strict I/O make hands-on care burden easier to distribute.
- **Infection prevention support:** isolation, Foley, CHG, line/drip, and high-care tags can keep infection-prevention workload visible during assignment building.
- **Failure-to-rescue risk:** acuity stacking, high patient counts, and fragmented handoff can make subtle deterioration harder to recognize. The platform helps identify stacked risk before the shift starts.
- **Handoff reliability:** fewer report sources and clearer ownership can reduce missed details during shift change.

These are operational risk-reduction mechanisms. They should be validated with local measures such as staff surveys, assignment-change time, falls, pressure injuries, CAUTI/CLABSI-related process measures, delayed response events, and handoff defects.

Evidence anchors

CDC reports that on any given day about 1 in 31 hospital patients has at least one healthcare-associated infection, supporting the need for reliable infection-prevention workflows and visibility. Source: <https://www.cdc.gov/healthcare-associated-infections/php/data/>

Aiken et al. found that each additional patient per nurse was associated with a 7% increase in odds of 30-day mortality and a 7% increase in odds of failure-to-rescue among surgical patients, and was also associated with burnout and job dissatisfaction. Source: JAMA, "Hospital Nurse Staffing and Patient Mortality, Nurse Burnout, and Job Dissatisfaction" (2002), doi:10.1001/jama.288.16.1987

